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Name: كريمة صالح عبد التواب علي National ID: 27609090103985	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
 وزارة الداخلية
 إدارة الأحوال المدنية
 كريمة
 صلاح عبد الوهاب طه
 في منطقة ١٧ شبراخيت - محافظة البحيرة
 تاريخ ٢٠٠٩/٠٩/٠١
 الرقم ٨٥
 KH5903826

٢٠٢٣/٢
 ٢٧٨١٩٠٨٠١٠٣٩٨٥
 ربة منزل
 انش
 محمد بكرى
 المنطقة ١٧ شبراخيت - محافظة البحيرة
 ٢٠٠٩/٠٩/٠١
 ٨٥



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي للصحة النسائية
وزارة الصحة والسكان



Patient ID:
Name: كريمة صالح عبدالنواب علي
Number: 27609090103985

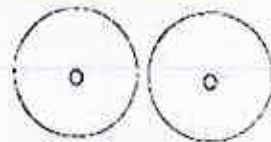
Sex:

MDT Form
2- Medical number:
Female

83252318
Old Med:

Old Med:
4601-2024

Patient Age: 48 Y



Family history: Family history
Phone: 91115409618
BMI: BMI
NRS Score: NRS Score
GROUP: GROUP
Menopausal status: Menopausal status
Initial STAGE: Initial STAGE

REASON OF MDT

Date of MDT

2025-04-19

Imaging:
regular speculated mass 13x2 cm at 2 o'clock
1 o'clock architectural distortion
Multiple enlarged axillary LN largest 4 cm

Clinical data

Mets Workup:
CTCAP FREE

ER 8/8 ER COMMENT:	PR 7/8 PR COMMENT:	Her 2 Neu 3 Her 2 Neu COMMENT:	SISH SISH COMMENT:	Ki67 >20
T4D	N 2	M 0 BY CT		

Mammography

PATIENT HISTORY

LABC, received NACT AC-->taxotere+phegso with good clinical response

MDT OUTCOME & PLAN

OR CT CAP WITH CONTRAST (IF FREE PROCEED) FOR LEFT MRM
EANWHILE CONTINUE ON PHEGSO AND ADJUVANT RADIATION

SIGNATURE

MEDICAL ONCOLOGY	RADIOLOGY
URGENT	PATHOLOGY
Psychiatric	Nutrition



Patient Name : الأستاذة / كريمه صالح عبد الكوابى عطى
Patient No. : 27609090103985
Age : 48 Y
Sex : Female
Referring Doc :

Request No. : 13002592
Request Date : 06/05/2025
Verification Date : 25/05/2025
Financial Category : اورام صدر المسلام - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.
Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

Procedure: Left MRM measured 28x23x8 cm with covering skin ellipse measured 28x23 cm.

Tumor extent: A rubbery greyish white tumor bed site.

Size: 12 x10 x6 cm.

Skin involvement: Nipple & areola were unremarkable grossly.

Distance from margins: 2 cm away from deep margin.

Lymph nodes number and size: Attached axillary fat measured 8x8 cm, entangling 11 nodes, measured up to 1.5x1 cm.

Designation:

10: Tumor bed site.

11: Nipple + deep margin.

12-14: Lymph nodes.

Dissected by: Dr. Nermin Mohamed.

Microscopic Examination:

Histological type: Minimal residual invasive duct carcinoma.

Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

Maximal tumor diameter 3 cm.

Actual tumor cellularity: 3%, tumor necrosis index: 97%.

Ductal carcinoma in situ: Not detected.

Skin involvement: Free nipple & areola.

Lymph node involvement: Positive lymph node metastasis in (1/11) lymph nodes.

Resection Margins: Free deep surgical resection margin.

Histologic response to therapy: Pathological partial response to therapy.

Verified by :

Dr.Toka Medhat

Printing Date : 27/05/2025 10:15:45

Page 1 of 2

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غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

atient Name : الأستاذ / كريمه صالح عبد التواب على
atient No. : 27609090103985
ith No. : 1617/25
ax : Female
ge : 48 Y
aferreing Doc :

Request No. : 13002592
Request Date : 06/05/2025
Verification Date : 25/05/2025
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Microcalcifications: Positive.

agnosis:

Left breast cancer post NACT, MRM specimen:
Minimal residual invasive duct carcinoma grade II, with microcalcifications.
Maximal tumor diameter: 3 cm.
Actual tumor cellularity: 3%, tumor necrosis index: 97%.
Free deep surgical resection margin.
Free nipple & areola.
Positive lymph node metastasis in (1/11) lymph nodes.
Pathological partial response to therapy.
Stage: ypT2, ypN1.
RCB: 3.132, RCB class II.

Signature
Dr. Rana Fathy
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

Printing Date : 27/05/2025 10:15:45

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Karima Saleh Abd El Tawab	Patient ID: 1231747
Dear. Sir:	Study Date: 29/04/2025
Acc No: 2149793	

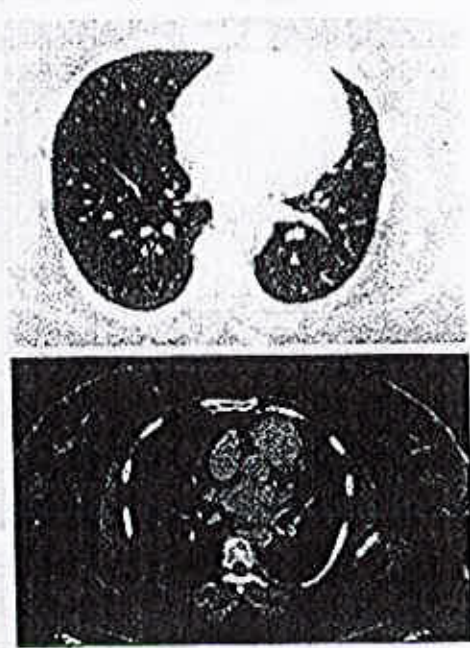
MULTISLICE C.T. SCAN OF THE CHEST, ABDOMEN AND PELVIS:

Multislice post contrast CT scan of the chest, abdomen and pelvis with multiplanar reformatted images revealed:-

CT CHEST FINDINGS:-

A 48-year-old female, known case of left breast cancer on treatment, coming for follow-up. In comparison to the previous PET CT study dated 04-01-2025, the current study reveals:

- Diffuse mosaic attenuation pattern of both lung fields is noted, suggesting an element of small air way disease versus underling pulmonary vascular pathology, for clinical correlation.
- Minimal interstitial pulmonary fibrotic changes are seen in the form of bilateral lower lobar posterior subpleural reticular infiltrates as well as fine fibroatelectatic bands.
- Otherwise, no evidence of sizable pulmonary nodules, masses, consolidation patches or cavitation.
- Regressive course regarding the previously reported left breast diffuse skin thickening as well as edematous changes of subcutaneous fat planes as well as breast parenchyma.
- Regressive course regarding the size of the previously reported left breast upper outer quadrant suspicious looking mass lesion, currently almost non detectable within the fibroglandular tissue of the breast, roughly measuring 3.5x2cm at maximum axial dimensions, for sonomammographic correlation.
- Regressive course regarding the size pf previously noted multiple metastatic left axillary lymph nodes, the largest currently



Patient Name: Karima Saleh Abd El Tawab	Patient ID: 1231747
Dear, Sir:	Study Date: 29/04/2025
Acc No: 2149793	

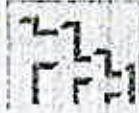
measuring 13x10mm associated with mild stranding and haziness of its surrounding axillary fat planes, for ultrasound correlation.

- Patent tracheo bronchial tree.
- No pleural effusion seen.

CT PELVI-ABDOMEN FINDINGS:-

- The liver is diffusely enlarged measuring 24 cm at maximum CC dimension, displaying a homogenously reduced CT density suggestive of underlying fatty infiltration with no definite focal lesions or dilatation of intra hepatic biliary radicles detected.
- The portal vein is patent with no thrombosis.
- The spleen is enlarged measuring 14cm at its longest axis, with homogenous CT density and no definite focal lesions.
- A small splenule is seen near the splenic hilum measuring 12mm in diameter.
- Bulky left adrenal gland is still seen with tiny calcific focus noted at its medial limb, yet with no sizable related masses could be seen, for clinical evaluation.
- Normal size and CT appearance of the right adrenal gland.
- Normal CT appearance of the pancreas.
- Both kidneys are functioning and normal.
- The urinary bladder is in adequately failed yet no stones or masses seen.
- Normal CT appearance of the uterus with no adnexal masses.
- Normal ischeorectal fossae on both sides.
- No abdominal or pelvic lymphadenopathy.





Age	48	Code	930-24
Date of receiving specimen	16-11-2024		
Sex	Female		
Left breast suspicious mass at 1 o'clock , BIRADS IVb .			
Core biopsy			

PATHOLOGY REPORT

Gross picture: Core biopsy specimen, totally embedded.

Microscopic Picture: Sections examined reveal tissue cores infiltrated by malignant cells arranged in clusters with moderate tendency to form tubules. The neoplastic cells show moderate degrees of anaplasia. No in-situ component seen in this biopsy.

Diagnosis: Left breast mass lesion; US guided biopsy:

- Invasive duct carcinoma, Grade II.

Prof. Dr. Mohamed Sherif, MD

Name: Karima Saleh Abd El Fawah	Patient ID: 1234567
Sex: Female	Study Date: 04/01/2025
Age: 48	

F18- FDG PET/CT EXAMINATION

History:-

48 years old female patient presented with recently diagnosed left breast invasive ductal carcinoma grade II. Referred for further PET/CT assessment.

Procedure:-

- 10.3 mCi ¹⁸F-FDG was administered to the patient and imaged after appropriate time.
- The blood glucose level was 201 mg/dl.
- A low dose non-contrast CT for attenuation correction and anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- A diagnostic 128 MDCT post contrast examination was taken after IV non-ionic contrast administration of the same regions. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.

PET/CT FINDINGS:-

A) Head and Neck:-

- Insignificantly avid left lower deep cervical/supraclavicular lymph node measuring 16.5 mm and achieving up to 3.27 SUV max.
- The brain exhibited normal FDG biodistribution. Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx. No parenchymal thyroid focal lesions could be elicited. Diffuse enlargement of the thyroid gland with coarse texture for sonographic correlation.



Dr. [Signature]
Radiologist
Date: 04/01/2025